



CORE CONTENT

Principles for Optimal Cancer Care

FINAL FOR CONSULTATION

20 July 2026

Contents

Principles for Optimal Cancer Care	2
Equity	3
Priority populations	3
Equity and optimal care	3
Person-centred care.....	5
Elements of person-centred cancer care	5
Safe and quality care	8
Considerations for safety and quality in cancer care	8
Multidisciplinary care.....	11
Principles of multidisciplinary care.....	11
Also consider	11
Multidisciplinary team membership	11
Networked care	12
Implementing multidisciplinary meetings	12
Supportive care	14
Domains of supportive care	14
When to assess supportive care needs.....	16
Sources of support and information.....	17
Navigation and care coordination.....	18
Components of navigation and care coordination	18
Sources of navigation and care coordination support.....	19
Communication	21
Elements of communication in cancer care	21
Formats and considerations for communication in cancer care.....	22
Research and clinical trials	24
Types of research relevant to cancer care	24
Clinical trials	24
Find out more	26
Unconscious bias	26
Culturally safe care.....	26
Identification of Aboriginal and Torres Strait Islander people	26
Co-design	26
Informed consent.....	27
Goals of care	27
PROMs and PREMs.....	27
Quality and safety standards	28
Treatment protocols	28

Cancer data	28
Networked care	29
Multidisciplinary care	29
Supportive care	29
Navigation and care coordination	31
Communication	31
Research and clinical trials.....	33
References	35

Principles for Optimal Cancer Care

The Principles for Optimal Cancer Care set the foundations for delivery of information, care and support for everyone who is at risk of and affected by cancer.

Principles are relevant to all cancer types and all populations.

What the principle means	 Equity	Considerations for delivery
	 Person-centred care	
	 Safe and quality care	
	 Multidisciplinary care	
	 Supportive care	
Key elements	 Navigation and care coordination	Population-specific considerations*
	 Communication	
	 Research and clinical trials	
Find out more Useful links and resources Including population-specific resources*		See also Links to other Principles and OCP pathway steps

Cancer-specific and population-based OCPs provide further detail about application of the principles at relevant pathway steps.

*Developed by Population-based OCP developers

Developers of population-based OCPs can use the Population-based OCP template to provide population-specific additions to the Principles for Optimal Cancer Care



Equity

Health equity in cancer outcomes is the absence of disparities for people with different levels of social advantage.¹

Achieving equity requires action to empower health services, systems and communities to recognise and respond to social, cultural, commercial, and environmental determinants of health that impact access to care.²

Priority populations

The Australian Cancer Plan highlights and aims to address disparities in cancer outcomes that exist between defined groups of the Australian population.³ Intersectionality can compound inequities⁴ and it is important to recognise that for many people, overlapping factors influence their cancer risk, diagnosis, treatment access and outcomes.

<u>Aboriginal and Torres Strait Islander people</u>	<u>People in lower socioeconomic groups</u>
<u>People living in rural and remote areas</u>	<u>People living with a mental illness</u>
<u>Lesbian, gay, bisexual, transgender, queer, intersex and asexual (LGBTQIA+) people</u>	<u>Older Australians</u>
<u>People from culturally and linguistically diverse backgrounds</u>	<u>Adolescents and young adults</u>
<u>People living with disability</u>	<u>Children</u>

Equity and optimal care

Equity must be embedded in the delivery of care across the cancer continuum. To improve cancer experience and outcomes, equity must be embedded in the delivery of care across the cancer continuum. This includes providing tailored, appropriate, and adequately resourced services to all Australians in need – when, where, and how they need them.

To enable equitable delivery of optimal cancer care and meet the needs of priority populations, primary, secondary and tertiary health services and the community sector need to work together across geographic areas and public and private services.



Key actions for equitable cancer care

- ✓ Services should collect and review data to understand disparities in cancer outcomes and their drivers to inform planning and delivery of equitable cancer care.⁵
- ✓ People working at all levels of a health service should undertake training in and maintain a sustained focus on unconscious bias and institutional racism^{6,7} to ensure that people do not experience disparities in access or care due to:
 - age
 - ethnicity
 - physical or cognitive disability
 - gender
 - sexuality
 - cultural or religious factors.
 - geography
 - language
- ✓ People working at all levels of the service should undertake training in and maintain a sustained focus on cultural competence knowledge, awareness, behaviour, skills and attitudes.^{8,9}
- ✓ People receiving cancer care should be asked how they identify to enable delivery of personalised and culturally safe care.¹⁰ How people identify must not influence the quality of care provided.
- ✓ Services should be delivered with due consideration of the financial impact of cancer, its diagnosis and treatment, with information and support provided to reduce the risk and impact of financial toxicity.¹¹
- ✓ Services delivering cancer care should utilise telehealth¹² and flexible service models to enable access to services and support appropriate to the person's location and circumstances.
- ✓ Services should enable inclusive participation in appropriate clinical trials and research.
- ✓ Services should apply co-design principles to ensure that services meet the needs and priorities of communities they are designed for.¹³
- ✓ Services delivering cancer care should have structures, services and environments appropriate to the age of people receiving care.^{14,15,16}
- ✓ All cancer care professionals should recognise the prevalence of trauma and be sensitive to and respectful of how past or ongoing trauma may impact a person's engagement with health services and cancer care. This extends to the person's family, carers and community.¹⁷



Find out more

[Resources about unconscious bias](#)
[Resources about culturally safe care](#)
[Resources about identification](#)
[Resources about co-design](#)



See also

- Considerations for specific population groups:
 - Optimal care pathway for Aboriginal and Torres Strait Islander people with cancer [<hyperlink to be added>](#)
 - Optimal care pathway for adolescents and young adults with cancer [<hyperlink to be added>](#)
 - Optimal care pathway for older people with cancer [<hyperlink to be added>](#)
 - [<Other population-based OCPs to be added as published>](#)



Person-centred care

Person-centred care means informing and involving people in their care, and respecting and responding to their individual needs, preferences and values, as well as those of their families and carers.¹⁸

Person-centred care improves the experience for everyone involved, strengthens the quality and safety of care, and reduces the cost of care.¹⁹

Person-centred care assumes an individual has capacity to make decisions for themselves. Where this is not the case, the multidisciplinary team should identify a substitute decision-maker who understands the person's wishes and who is empowered to make decisions on behalf of the person under state / territory law.

Elements of person-centred cancer care

	Understand what matters and deliver care that responds to individual needs and preferences
	Discuss and agree goals of care
	Respect and respond to complex needs
	Enable informed choice and informed consent
	Use digital health appropriately
	Enable shared care
	Practise family-, carer and community-centred and inclusive care

<Additional information boxes linked from points above>

	Decisions about cancer care are not only influenced by cancer type, stage of disease and other health conditions. Factors such as age, stage of life, culture, gender, and where a person lives and works, shape what is important for people with cancer and their families and carers. Delivering care that responds to individual needs and preferences includes: • challenging <u>unconscious bias</u>, <u>racism</u> and <u>ageism</u> • asking people how they identify • practising <u>culturally safe</u>,^{20,21} <u>trauma-informed care</u> • considering <u>place-based approaches</u> to care.
--	--



Discuss and agree goals of care

Goals of care for cancer include:

- treatment intent (e.g. curative vs symptom control)
- personal goals (e.g. being well enough to work, travel or participate in an important family event)
- consideration of how the person wants to engage in healthcare.²²

Goals of care can change over time and require review at key transition points.



Enable informed choice and consent^{23,24}

People have the right to understand what is involved in any test, procedure or intervention related to cancer care.

Health professionals must enable informed choice and are legally required to obtain informed consent from the person, or their substitute decision-maker, before any test, procedure or intervention.

Informed choice and informed consent apply to:

- options, benefits and risks of tests, procedures and interventions
- financial costs of tests, procedures and interventions
- what health and personal information will be collected and how it will be used
- genetic testing
- participation in research.



Use digital health appropriately^{25,26}

The growth of digital technology in cancer care brings challenges and opportunities for person-centred care.

Health professionals should consider the role that telehealth and other forms of digital health can play in enabling person-centred care. In doing so it is important to understand people's access to and confidence in using digital technology as part of their care.



Enable shared care²⁷

Shared care supports person-centred care by providing care closer to home, reducing duplication, and involving health professionals involved in other aspects of a person's healthcare.

Care can be shared between a cancer specialist and primary care provider, between different cancer specialists, or between cancer specialists and a nurse or allied health professional. Family members and carers can also play an active role in shared care.



Practise family-, carer- and community-centred / inclusive care^{28,29}

For many people, person-centred care means care that is inclusive of family and carers. For some people, particularly Aboriginal and Torres Strait Islander peoples, this extends to community.

Family, carers and community provide important support for people across the cancer care continuum and have their own information and support needs.

Health professionals should proactively integrate family, carers and community into cancer care to the extent the person receiving cancer care wants them involved.



Key actions for delivering person-centred care^{30,31}

- ✓ Involve the person in decisions about their cancer care
- ✓ Involve family and carers to the extent the person would like them involved
- ✓ Find out what is important to the person, their families and carers and tailor communication and care accordingly, with consideration to:
 - life stage, experiences, culture and beliefs
 - literacy, health literacy, digital literacy and preferred language
- ✓ Provide timely and accessible information to enable informed choices about cancer care according to individual preferences and goals, including information about:
 - all options for tests or treatments and associated advantages and disadvantages
 - where care will be delivered and likely waiting times
 - timeframes in which decisions are needed
 - costs, including public and private options and out-of-pocket expenses
- ✓ Discuss and agree goals of care and revisit these at key transition points
- ✓ Discuss options and preferences for shared care
- ✓ Use patient-reported outcome measures (PROMs) and patient-reported experience measures (PREMs) to understand and respond to peoples' needs and preferences³²
- ✓ Obtain informed consent for all tests, procedures and interventions from the person or their substitute decision-maker³³



Find out more

[Resources about informed consent](#)
[Resources about goals of care](#)
[Resources about unconscious bias](#)
[Resources about culturally safe care](#)
[Resources about identification](#)
[Resources about PROMs and PREMs](#)



See also

- Equity (Principle) [<hyperlink to be added>](#)
- Communication (Principle) [<hyperlink to be added>](#)
- Supportive care (Principle) [<hyperlink to be added>](#)
- Research and clinical trials (Principle) [<hyperlink to be added>](#)
- Advance care planning (Pathway step 6) [<hyperlink to be added>](#)
- Population-based OCPs [<hyperlink to be added>](#)



Safe and quality care

Safe and quality care means delivering the most appropriate and best-value care, while keeping people safe from preventable harm in the delivery of that care.³⁴

Delivery of safe and quality cancer care is influenced by:

- **individual factors**, such as patient age and stage of life
- **clinical factors**, such as stage of disease and co-morbidities
- **service factors**, such as the setting in which care is delivered
- **system factors**, including national and state/territory policies and regulations and approved and reimbursed tests and treatments.³⁵

Safety and quality at all levels should include clinical considerations alongside issues of bias, cultural safety, racism and other factors that may compromise how a person accesses or experiences care.

Considerations for safety and quality in cancer care



Deliver cancer care by **appropriately trained and credentialled health professionals**



Ensure appropriate **capability and staffing of health services** providing cancer care



Deliver **evidence-based care**



Deliver **data-informed care**



Enable and undertake **research to drive quality improvement**



Use **digital health and technology** appropriately

<Additional information boxes linked from points above>



**Trained and
credentialled health
professionals**

Cancer care should be provided by health professionals with training and experience appropriate to the cancer type and individual characteristics such as age, stage of life, co-morbidities, culture and language.

Individual OCPs contain details of specific training required for the delivery of care for specific cancer types and population groups.



**Health service
capability**

Health services providing cancer care must have equipment, staff, policies and procedures required to support safe, high-quality care.

While person-centred care aims to provide care as close to home as safely possible, some aspects of diagnosis and treatment can only be undertaken safely in specialist centres and may require the person to travel.

Where evidence demonstrates improved outcomes in high-volume or specialist centres, referral to appropriately experienced multidisciplinary services should be considered.

	<i>Individual OCPs contain details of specific training required for the delivery of care for specific cancer types and population groups</i>
 Evidence-based care	Cancer care should be informed by best available evidence with reference to: • national and international clinical practice guidelines • evidence-based treatment protocols • validated models of care. <i>Individual OCPs contain details of clinical practice guidelines, protocols and models of care relevant to cancer types and population groups.</i>
 Data-informed quality care	Services and health professionals delivering cancer care should collect, review and act on relevant data to inform evidence-based, high-quality care. Measuring capabilities and performance against the OCPs can identify improvements to drive better and equitable patient outcomes. Data sets relevant to cancer care include: • National Cancer Control Indicators³⁶ • Kulay Kalingka Aboriginal and Torres Strait Islander Cancer Control Indicators³⁷ • <u>Patient-reported outcome measures (PROMs)</u>³⁸ • <u>Patient-reported experience measures (PREMs)</u>³⁹ • Radiation oncology clinical indicators.⁴⁰ <i>Individual cancer-specific and population-based OCPs contain details of data sets relevant to cancer types and population groups.</i>
 Research	Research is an important driver of evidence-based care and can increase the quality and safety of care for people with cancer. <i>Individual OCPs contain details of research and clinical trial considerations at different pathway steps, relevant to cancer types and population groups.</i>
 Use of digital health and technology	Use of digital health and artificial intelligence (AI) is increasing across healthcare, leading to new considerations and associated standards for safety and quality. Examples include standards for: • <u>telehealth</u>^{41,42,43} • use of AI^{44,45,46,47,48} • use of social media.⁴⁹ AI and social media also present challenges in relation to misinformation for people at risk of and diagnosed with cancer.



Key actions for enabling safety and quality in cancer care

Actions for health professionals referring a person for cancer-related investigations, treatment or care

- ✓ Refer people to health professionals and health services with relevant skills and expertise
- ✓ Consider referral to another service if safe and high-quality treatment and care cannot be provided locally

Actions for health professionals providing cancer care

- ✓ Undertake regular quality reviews of performance
- ✓ Contribute to regular audits of care
- ✓ Actively engage in continuing professional development, including clinical and person-centred skills
- ✓ Maintain active membership of professional societies and organisations and participate in professional development
- ✓ Participate in research and quality improvement

Actions for health services delivering cancer care

- ✓ Embed clinical practice guidelines and protocols into local models of care to enable delivery of evidence-based care
- ✓ Use OCPs as a framework for monitoring and evaluating practice
- ✓ Support the collection and use of well-defined, validated, cancer- and age-specific PROMs and PREMs to inform optimal care
- ✓ Routinely collect and contribute to relevant minimum cancer-related datasets to support benchmarking, quality care and service improvement, including population-specific data to enable benchmarking and quality improvement in relation to equity and inclusion.
- ✓ Participate in audits
- ✓ Enable and encourage research and quality improvement



Find out more

[Resources about quality and safety standards](#)
[Resources about treatment protocols](#)
[Resources about cancer data](#)
[Resources about informed consent](#)
[Resources about cultural safety](#)
[Resources about PROMs and PREMs](#)



See also

- Individual OCPs contain details of training and credentialling, health service requirements and clinical practice guidelines relevant to specific cancer types and population groups.
- Equity (Principle) [<hyperlink to be added>](#)
- Communication (Principle) [<hyperlink to be added>](#)



Multidisciplinary care

Multidisciplinary care is an integrated team approach that involves all relevant health professionals discussing all relevant treatment options and making joint recommendations about treatment and supportive care plans that consider the individual characteristics and preferences of the person with cancer.⁵⁰

Multidisciplinary care improves outcomes for people with cancer, including survival.⁵¹

Principles of multidisciplinary care⁵²

Cancer Australia's [Principles of multidisciplinary care](#) provide a flexible definition of multidisciplinary care, allowing services to vary implementation according to cancer type and the service location and context.



Team approach involving core disciplines integral to the provision of good care, including general practice, with input from other specialties as required



Communication among team members regarding treatment planning



Access to the **full therapeutic range** for all patients, regardless of geographical remoteness or size of institution^a



Provision of care in accordance with **nationally agreed standards**



Involvement of patients in decisions about their care

Also consider

- Cancer Australia's [Multidisciplinary care principles for advanced disease](#), which aim to improve care and quality of life of patients with advanced disease while maximising comfort and functioning⁵³
- Cancer Australia considerations for [multidisciplinary care for people in regional cancer centres](#).

Multidisciplinary team membership

The multidisciplinary team should include core disciplines that are integral to providing good care. Team membership should reflect both clinical and supportive care aspects of care. Pathology and radiology expertise are essential.

Core members of the multidisciplinary team are expected to attend most multidisciplinary meetings either in person or remotely via virtual mechanisms. The person's GP is a core member of the team and should be actively engaged and informed from the point of diagnosis.

^a Where evidence demonstrates improved outcomes in high-volume or specialist centres, referral to appropriately experienced multidisciplinary services should be considered.



See also

- Individual OCPs contain details of multidisciplinary team membership for specific cancer types and population groups

Networked care

Australia has strong cancer outcomes overall, with survival rates for many cancers among the best in the world. However, cancer outcomes are not equal for all Australians. Disparities are influenced by many factors, including differences in access to cancer services, specialist expertise and multidisciplinary care across the country.

The [Australian Comprehensive Cancer Network \(ACCN\)](#) aims to address these disparities through an integrated national network that connects cancer services across Australia.

The ACCN brings together services, networks and alliances, Comprehensive Cancer Centres, and other specialist providers to strengthen collaboration, facilitate the sharing of expertise, and support access to high-quality, coordinated cancer care for all Australians



Find out more

[Resources about networked care](#)

Implementing multidisciplinary meetings

Multidisciplinary meetings are a core element of multidisciplinary care and should reflect the [Principles of multidisciplinary care](#).



Key actions for multidisciplinary team meetings

- ✓ Ensure appropriate attendance at the multidisciplinary meeting based on cancer type, stage and personal patient characteristics and include adequate administrative support
- ✓ Use telehealth / virtual care to enable networked access to multidisciplinary team expertise^{54,55,56}
- ✓ Maintain protocols for which cases will be presented / prioritised and how treatment plans for cases not discussed in detail are documented⁵⁷
- ✓ Consider medicolegal considerations, including patient consent and privacy, professional liability, recording of dissenting views, and duty of care⁵⁸
- ✓ Ensure minimum diagnostic tests, including genomic information where required, are available for discussion; do not delay presentation at the meeting if the full range of tests is not available
- ✓ Consider options that reflect stage of life, life expectancy, performance status and decision-making capacity, goals of care, values, beliefs and cultural needs of the person
- ✓ Consider factors that may influence a person's access to treatment such as requirement for travel, cost and carer responsibilities
- ✓ Develop and agree treatment recommendations based on treatment intent, including consideration of clinical trials
- ✓ Record the outcomes of the discussion, including decisions for any non-complex patients not discussed during the meeting

- ✓ Document outcomes and treatment plan in the patient's medical record history and communicate to the general practitioner



Find out more

[Resources about multidisciplinary care](#)



See also

- Multidisciplinary team membership for individual cancer types <Cancer-specific OCPs> [<hyperlink to be added>](#)
- Multidisciplinary team considerations for specific population groups: <Population-based OCPs> [<hyperlink to be added>](#)

Supportive care

Supportive care involves prevention and management of the adverse effects of cancer and its treatment, including management of physical and psychological symptoms and side effects across the cancer continuum. Supportive care aims to improve the quality of rehabilitation, secondary cancer prevention, survivorship, and end-of-life care.⁵⁹

Supportive care is an essential component of cancer care that requires multidisciplinary input and interventions. It is needs based and is important for people with cancer, their families and carers. Supportive care is important across all steps of the optimal care pathway. Definitions and frameworks have evolved over time, incorporating elements of survivorship care and palliative care.⁶⁰

Domains of supportive care

Supportive care for people with cancer, their families and carers can be considered in five key domains.

	Physical: physical symptoms and side effects that may be acute, relatively short-lived or ongoing, requiring continuing interventions or rehabilitation
	Psychological: issues related to the person's emotional and mental health wellbeing
	Social: social and practical issues that affect the person, their family and carers
	Informational: information needs about cancer across the care pathway
	Spiritual: relating to a person's changing sense of self and challenges to their underlying beliefs and existential concerns

<Additional information boxes linked from points above>

 Physical symptoms	Common physical symptoms associated with cancer include: • anaemia • ascites • cardiotoxicity • changes in appearance • decline in mobility • fatigue • fertility changes • fever • insomnia • nausea and vomiting • neutropenia • pain • weight loss or gain
 Psychological symptoms	Common psychological symptoms associated with cancer include: • anger • anxiety • changes in cognition • depression • fear • grief • guilt • shame



Social impacts

Common social impacts of cancer include:

- carer responsibilities
- impact on children
- impact on education
- finance
- intimacy / sexuality
- isolation
- legal impacts
- relationships
- travel
- working / return to work



Informational needs

People with cancer have informational needs across the care continuum and across all domains of supportive care.

Preferences for the amount and format of information vary and preferences may change at different times.

Carers and family members also require information and their needs and preferences may differ to those of the person with cancer.



Spiritual needs

Spiritual needs include but are not limited to:

- existential concerns and fears
- impact on faith
- impact on sense of self.

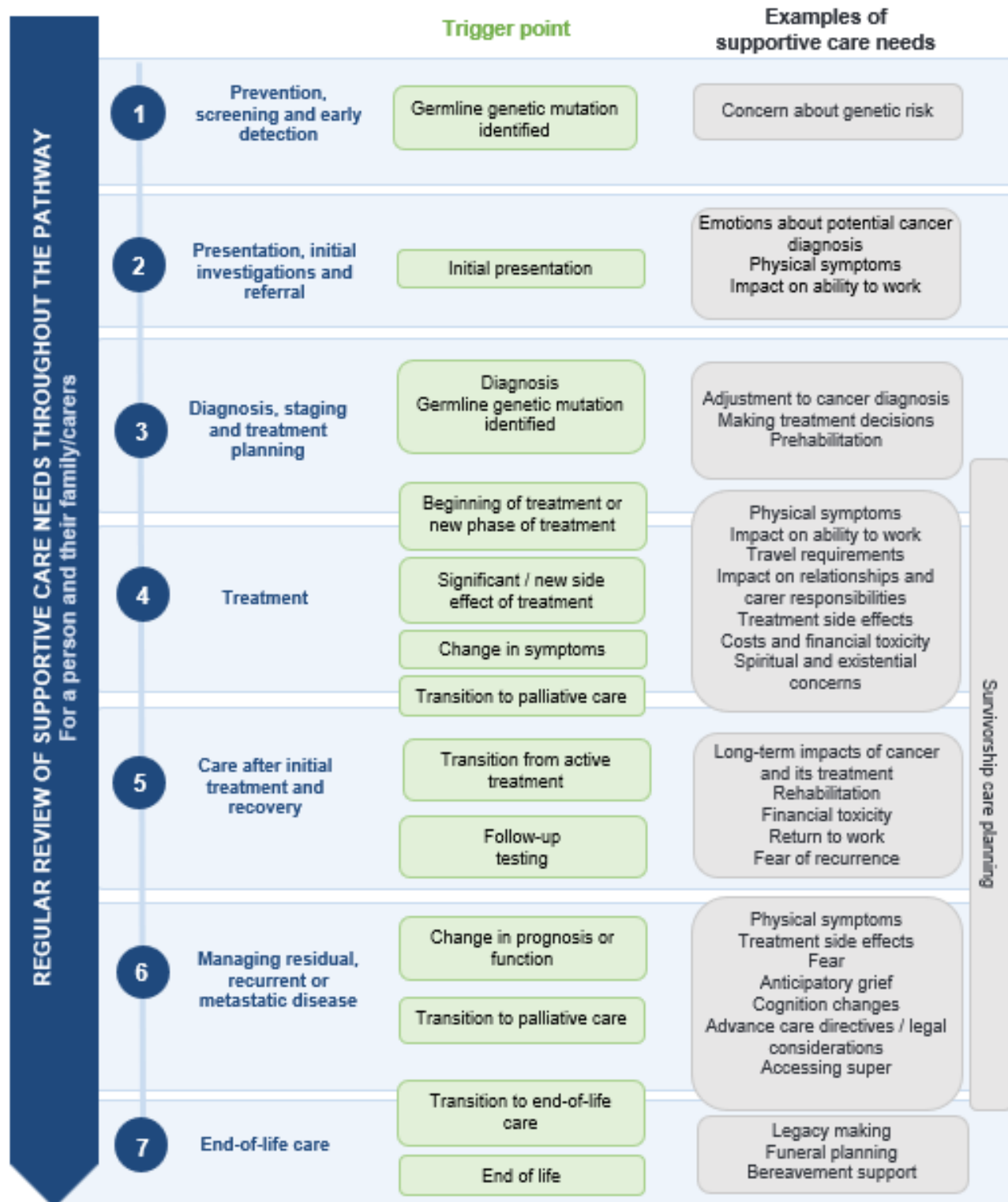


Key actions for implementing supportive care

- ✓ Embed supportive care screening into models of care across the care pathway with clearly defined responsibilities and strategies to act on the information collected
- ✓ Collect and use patient-reported outcome measures (PROMs) and patient-reported experience measures (PREMs)
- ✓ Assess and screen people and their families and carers to understand supportive care needs across each of the five domains
- ✓ Consider needs and tailor support that reflects individual as well as clinical considerations, including:
 - the additional burden of age, physical or cognitive disability, mental illness
 - factors related to life stage, including carer responsibilities
 - cultural considerations
 - barriers to accessing supportive care services including geography, cost or disability
 - carer, family and community needs
- ✓ Provide people with information to enable and empower them to articulate, understand, self-manage and seek support for their supportive care needs
- ✓ Refer people to appropriate services and organisations, including community and non-government organisations, who can provide timely support to optimise recovery
- ✓ Use evidence-based, multidisciplinary approaches to management of symptoms and side effects
- ✓ Provide information and skills training, as required, to people and their families and carers for home-based care

When to assess supportive care needs

Supportive care needs should be assessed at key transition points, and in response to clinical judgement and requests by the person with cancer and their families and carers.



Sources of support and information

All members of the multidisciplinary team have a role in providing supportive care along the care pathway. It is important to be aware that some sources of supportive care are not available in private health services. Others may only be available from private providers.

Examples of who can help and sources of information are provided below. Further detail is provided in cancer-specific and population-based OCPs.

Supportive care domain	Sources of support	
 Physical symptoms	✓ Dietitian ✓ Exercise physiologist ✓ Occupational therapist ✓ Physiotherapist	✓ Lymphoedema practitioner ✓ Palliative care teams ✓ Pain specialists
 Psychological symptoms	✓ Counsellor ✓ Psychologist	✓ Psychiatrist ✓ Peer support
 Social impacts	✓ Social worker ✓ Centrelink	✓ Peer support groups
 Informational needs	✓ Websites ✓ Non-government organisations	✓ Translation and Interpreter Services
 Spiritual needs	✓ Religious or faith-based leaders	✓ Community elders

 Find out more
Cancer support services Supportive care screening tools Supportive care protocols and guidelines Sources of practical and social support
 See also
• Individual OCPs contain details of supportive care needs in each domain associated with specific cancer types and population groups • Equity (Principle) <hyperlink to be added> • Communication (Principle) <hyperlink to be added> • Person-centred care (Principle) <hyperlink to be added> • Navigation and care coordination (Principle) <hyperlink to be added>



Navigation and care coordination

Navigation and care coordination are related person-centred interventions that aim to empower patients, families and carers affected by cancer to facilitate safe and timely access to optimal cancer-related care and support.⁶¹

Components of navigation and care coordination

Navigation and care coordination are essential across the care pathway and include a range of inter-related components.⁶² Needs should be assessed regularly and particularly at key transition points.

	Needs assessment enabling escalation or de-escalation of support
	Provision of education and information , including in-language assistance
	Connection and referral to appropriate clinical and supportive care
	Understanding and removing barriers to care
	Support for travel, accommodation and other logistics
	Financial information and assistance
	Enabling self-empowerment
	Advocacy

Navigation and care coordination should take account of the needs of the individual as well as clinical considerations for their cancer, including factors such as:

- age
- geography
- ethnicity
- sexuality
- language
- physical or cognitive disability
- cultural factors.



Key actions for enabling navigation and care coordination

- ✓ Communicate proactively and in a timely way with patients, their families and carers, using translation or interpreter services as required
- ✓ Use and act on insights from patient-reported outcome measures (PROMs), patient-reported experience measures (PREMs) and supportive care screening tools
- ✓ Use and share treatment plans, survivorship care plans and advance care directives

✓ Involve the person's primary care provider in care from the point of diagnosis and maintain regular and timely communication
✓ Participate in <u>multidisciplinary meetings</u>
✓ Hold multidisciplinary assessment clinics and / or coordinate patient appointments to streamline access and care
✓ Use <u>telehealth</u> and <u>virtual care</u> to connect people to care
✓ Enable shared access to medical records for all members of the multidisciplinary team
✓ Prioritise timely availability of test results to enable recommendations and decision-making

Sources of navigation and care coordination support

Everyone involved in cancer-related care has a role in navigation and care coordination, including health professionals and administrative staff in cancer services, primary care and community organisations.⁶³ Navigation should be:

- **integrated:** providing seamless, effective, and efficient care that is fit for purpose and person-centred
- **multi-channel:** using multiple communication channels and provider platforms that empower people to access information and support across multiple touchpoints
- **multidisciplinary:** involving health professionals, informal carers and relevant community support personnel.

People	Information	Tools and resources
Formal care coordinators or navigators (<i>state- and territory-specific</i>)	Written information	Digital or electronic tools integrated into electronic health records ³⁵
Informal carers	Call-centres, support lines and information hubs / portals	Mobile apps
Community support personnel	Email	Websites and search engines
	Social media	

Australian Cancer Nursing and Navigation Program (ACNNP)

The ACNNP is free to access and designed to help people affected by cancer, including families, friends and carers to navigate the health system, to find the information and support they need, when they need it. The program has three connected components.

Cancer Navigation Services

Cancer Navigation Service

The Cancer Council delivers navigation supports to help people impacted by cancer find high quality information and support, depending on their needs. Individuals can self-navigate using the Cancer Connect digital navigation platform, or access help to navigate available supports through the 13 11 20 phonenumber. Individuals can be connected with services delivered not only by Cancer Council but also the broader ACNNP and cancer care ecosystem. Referrals can be made to Cancer Council via this [form](#).

Cancer Hub

The Cancer Hub is a collaboration between Canteen, Red Kite and Camp Quality, to support children and young people ages 0–25, and families with children or young people, who are impacted by any cancer. Services

	include navigation and emotional support, such as case coordination, counselling and peer support. Cancer Hub can be contacted on 1800 431 312 and referrals can be made via this form .
Cancer Care Nursing Services	Cancer Care Nurse Service The McGrath Foundation is delivering Commonwealth-funded cancer care nurses in health and hospital services across Australia. Patients receiving cancer care in a region where a McGrath Nurse is placed have access to a range of supportive care types including practical, informational and emotional support. Contact the McGrath Foundation Nursing Program Team on 02 8962 6100 to understand if there is a nurse available in a particular area. Prostate Cancer Specialist Nurses The Prostate Cancer Foundation of Australia (PCFA) is also delivering specialist prostate cancer nurses around Australia. These nurses provide care and support to prostate cancer patients and their families across the cancer continuum. Contact PCFA on 1800 220 099 to find out if there is a nurse in a particular area.
Specialist Support Services	Twelve non-government organisations are funded to provide disease-specific information and support tailored to the unique needs of particular cancer types. Support is available via telephone and digital means for patients, families, carers, and friends. Health professionals may also access the services for disease-specific advice to support their patients. Services available from each organisation differs depending on specific needs of the individual communities.
Referrals	Referrals can be made to any of the above services by health professionals on behalf of patients, or directly by the individual requiring support. Health professionals and consumers can contact the relevant organisation(s) directly using the details and links above. Support can be accessed from more than one organisation at any given time, depending on the needs of the individual. Where a health professional or consumer is unsure what support is available or where to find support, they should contact Cancer Council on 13 11 20.

 Find out more
Resources about navigation and care coordination
 See also
• Communication (Principle) <hyperlink to be added> • Equity (Principle) <hyperlink to be added> • Person-centred care (Principle) <hyperlink to be added> • Multidisciplinary care (Principle) <hyperlink to be added>

Communication

Good communication is an essential part of cancer care and a responsibility of everyone involved in healthcare across the cancer continuum.⁶⁴

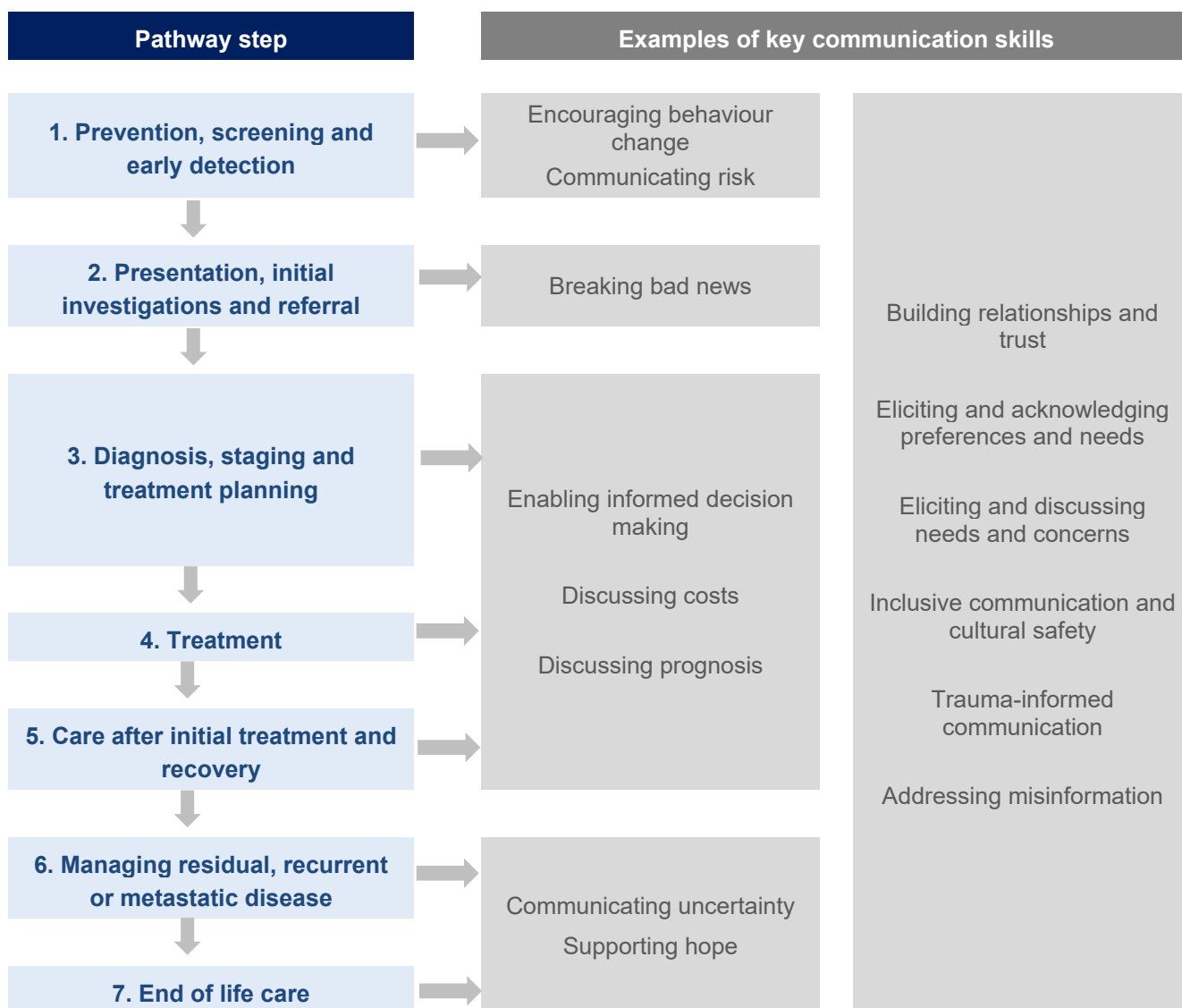
Delivery of optimal cancer care requires high-quality, consistent and timely communication.

This Principle refers to communication between health professionals and people at risk of or diagnosed with cancer, their families and carers. Communication between health professionals is covered in sections on Shared care and Navigation and care coordination.

Elements of communication in cancer care

Communication with patients, families and carers must take account of the individual needs and preferences, with respect for age, culture, language, hearing or visual impairments and literacy.

Communication in cancer care covers a breadth of topics and requires skills, practice and judgement.⁶⁵



Formats and considerations for communication in cancer care

A range of considerations influence approaches to communication in cancer care.⁶⁶ It is important not to make assumptions about communication preferences.

	In-person	Face-to-face consultation should be the first option when delivering critical diagnosis information, a change in therapy or prescribing intensive treatment. If face-to-face is not an option, consider video consultation and encourage the patient to have a support person present⁶⁷
	Telehealth	Video and telephone are effective forms of communication with people with cancer⁶⁸ When considering options for telehealth, it is important to consider individual preferences, access to and familiarity with digital communication, and any sensory or cognitive issues⁶⁹
	Digital and AI-enabled communication tools (clinician focused)	Digital and AI-enabled tools are increasingly used to support communication and documentation in cancer care. Examples include consultation recording, ambient scribing, note-taking, transcription and summarisation tools. These technologies may reduce administrative burden and support communication between health professionals, patients, families and carers. Health professionals should be transparent about how these tools are being used, ensure appropriate consent and privacy protections, and verify the accuracy of information generated.
	Health literacy	Health literacy – and broader literacy – are influenced by age, education and learning opportunities, disability and access needs, culture and language⁷⁰
	Digital literacy (person-focused)	High digital literacy can improve communication and empower people to manage their care. However, use of digital technologies with people who have low digital literacy can widen disparities. It is important to consider patient and carer confidence in using digital technologies.
	Language	A professionally trained health interpreter (not a family member or friend) should be made available when communicating with people with limited English proficiency. Navigation of the Australian healthcare system can pose problems for those with a non-Anglo culture, and treatment teams should pay particular attention to supporting these patients.



Key actions for enabling communication

- ✓ Ask people about how they like to communicate, including:
 - who they would like to be present during consultations
 - assistance needed due to visual or hearing impairment
 - interpreter support
 - considerations for literacy or other factors influencing information comprehension or retention
- ✓ Encourage and support the involvement of family members and carers in appointments
- ✓ Identify whether a person has a substitute decision-maker and involve them in relevant discussions

✓ Use plain language and avoid jargon and complex medical terms
✓ Use diagrams and validated tools and / or decision aids (see examples) <hyperlink to Find out more to be added>
✓ Provide written information and / or links to digital sources of information including videos
✓ Consider providing a question prompt list before appointments and offering a recording of the discussion to assist with recall (see examples) <hyperlink to Find out more to be added>
✓ Encourage people to ask questions and voice concerns
✓ Check for understanding by asking the people to relay what you have discussed in their own words
✓ Be truthful and transparent but sensitive to the psychological impact of information being conveyed
✓ Allow time appropriate to the level of complexity and sensitivity of the discussion and decisions
✓ Seek consent before sharing information between healthcare providers or with families or carers
✓ Make efforts to involve people in discussions about their care, to the extent they are able, even if decision-making has been transferred to a <u>substitute decision-maker</u>
✓ Be respectful if a person asks for a second opinion
☒ Do not make assumptions about a person's communication preferences
☒ Do not expect patients, their families or carers to convey information between health professionals
☒ Do not rely on family members to act as interpreter unless there is no alternative
☒ Do not use stigmatising language or reinforcing stereotypes

 Find out more
Resources about health literacy Resources about communication in other languages Resources about digital communication and AI Resources about communication skills Resources about carer communication
 See also
• Specific cancer-related and population-based considerations for communication are provided in individual OCPs • Equity (Principle) <hyperlink to be added> • Person-centred care (Principle) <hyperlink to be added>



Research and clinical trials

Research and clinical trials generate evidence to improve understanding of cancer and to accelerate progress in prevention, detection, diagnosis, treatment, supportive care, survivorship care, palliative and end-of-life care.⁷¹

Types of research relevant to cancer care⁷²

Clinical trials are the foundation for generating evidence to improve cancer outcomes. They are used to test a range of interventions including:

- diagnostic tests
- drug interventions
- supportive care interventions
- symptom management
- pain control
- survivorship care
- health service interventions.

Clinical trials can provide options for cancer patients when other therapies have failed or are yet to be approved. For some cancer types, clinical trials are standard of care and may be a first-line treatment option.

Other types of clinical research include:

- observational studies
- qualitative research
- implementation studies
- health services research.

Clinical trials

Key considerations about clinical trials
• Clinical trials are not available for all cancer types, stages of cancer or population groups
• Compassionate access schemes can be an alternative to accessing drug if a clinical trial is not available
• <u>Informed consent</u> for research participation and for data and tissue collection and storage are essential – The decision to join a suitable trial is always the patient's to make
• Access to trials is an important consideration: – travel and accommodation support are increasingly available to support participation for people who need to travel to participate in a trial – decentralised trials and teletrials provide access to trials closer to home



Key actions regarding clinical trial participation

- ✓ Look for suitable clinical trials for people with cancer and remain vigilant for opportunities based on a person's changing medical status and diagnostic profile
- ✓ Discuss the benefits of clinical trial participation with patients, their families and carers
- ✓ Actively support participation of people with cancer in suitable trials, even if this requires referral to another service or centre
- ✓ Continue to provide comprehensive care for people on clinical trials to ensure consistency in navigation and provision of supportive care
- ✓ Be ready to acknowledge disappointment and provide support if a suitable trial is not available
- ✓ Consider referral to supportive care services to help people to navigate the emotional impact of a suitable trial not being available, particularly if treatment options are limited
- ☒ Do not make assumptions about who is and is not suitable for a clinical trial



Find out more

[Resources about clinical trials](#)

[Resources about decentralised trials and teletrials](#)

[Resources about interpreters in research](#)



See also

- Equity (Principle) [<hyperlink to be added>](#)
- Informed consent – in Person-centred care (Principle) [<hyperlink to be added>](#)

Find out more

Unconscious bias



Find out more

Australian and New Zealand College of Anaesthetists & Faculty of Pain Medicine Unconscious bias toolkit Toolkit to help health professionals understand, identify and mitigate the effects of unconscious bias <hyperlink https://www.anzca.edu.au/getContentAsset/4d71a9b8-5203-464f-a850-5339eb713906/80feb437-d24d-46b8-a858-4a2a28b9b970/ANZCA_GE-unconscious-bias_2023.pdf?language=en&view=1>

Culturally safe care



Find out more

Australian Health Ministers' Advisory Council's National Aboriginal and Torres Strait Islander Health Standing Committee Cultural Respect Framework 2016-2026 for Aboriginal and Torres Strait Islander health Framework for use by the health departments and hospital and primary health care settings to guide strategies to improve culturally respectful services <hyperlink https://healthinfonet.ecu.edu.au/key-resources/policies-and-strategies/32498/?title=Cultural+Respect+Framework+2016+2026+for+Aboriginal+and+Torres+Strait+Islander+health&contentid=32498_1>

Australian Commission on Safety and Quality in Health Care National Safety and Quality Health Service Standards User Guide for Aboriginal and Torres Strait Islander Health Action 1.2 Addressing health needs of Aboriginal and Torres Strait Islander people <hyperlink <https://www.safetyandquality.gov.au/sites/default/files/resources/attachments/National-Safety-and-Quality-Health-Service-Standards-User-Guide-for-Aboriginal-and-Torres-Strait-Islander-Health.pdf>>

Australian Commission on Safety and Quality in Health Care National Safety and Quality Health Service Standards User Guide for Aboriginal and Torres Strait Islander Health Action 5.8 Identifying people of Aboriginal and / or Torres Strait Islander origin <hyperlink <https://www.safetyandquality.gov.au/sites/default/files/resources/attachments/National-Safety-and-Quality-Health-Service-Standards-User-Guide-for-Aboriginal-and-Torres-Strait-Islander-Health.pdf>>

Identification of Aboriginal and Torres Strait Islander people



Find out more

Action 5.8 of the National Safety and Quality Health Service Standards Focuses on identifying people of Aboriginal and Torres Strait Islander origin: <hyperlink: www.safetyandquality.gov.au/sites/default/files/migrated/National-Safety-and-Quality-Health-Service-Standards-User-Guide-for-Aboriginal-and-Torres-Strait-Islander-Health.pdf>

Co-design



Find out more

Cancer Australia Guide to co-design with Aboriginal and Torres Strait Islander people

Guidance about using best practice co-design approaches that enable Aboriginal and Torres Strait Islander people to take the lead in determining priorities and shaping the course of action. <hyperlink <https://www.canceraustralia.gov.au/guide-co-design-aboriginal-and-torres-strait-islander-people>>

Australian Commission on Safety and Quality in Health Care Co-design and consumer engagement Guides and resources to help health professionals in co-designing meaningfully with consumers on the design of health services <hyperlink <https://www.safetyandquality.gov.au/clinical-topics/person-centred-care/co-design-and-consumer-engagement>>

Informed consent

Find out more

Australian Commission on Safety and Quality in Healthcare Informed consent in healthcare Fact sheet for health professionals with guidance about obtaining informed consent <hyperlink <https://www.safetyandquality.gov.au/resources/informed-consent-practical-guidance-clinicians>>

Cancer Council Australia Standard for informed financial consent Guidance for health professionals about how to enable patients to better consider the likely financial impact of cancer care <hyperlink <https://www.cancer.org.au/health-professionals/resources/informed-financial-consent>>

Genomics Australia Consent for genomic testing Guidance for health professionals about obtaining informed consent for genomic testing <hyperlink <https://www.genomicsaustralia.gov.au/healthcare-professionals/consent-genomic-testing>>

Office of the Australian Information Commissioner Collecting health information Guidance for health professionals about collecting health information from patients <hyperlink <https://www.oaic.gov.au/privacy/privacy-guidance-for-organisations-and-government-agencies/health-service-providers/guide-to-health-privacy/chapter-2-collecting-health-information>>

Cancer Australia Informed consent for participation in research Guidance for health professionals about enabling people to make an informed choice about participating in clinical trials <hyperlink <https://www.canceraustralia.gov.au/consumer-learning/informed-consent>>

Goals of care

Find out more

Australian Commission for Safety and Quality in Healthcare Identifying goals of care - Tips for clinicians Eight tips to assist patients and family, carers and other support people to identify and develop goals of care <hyperlink <https://www.safetyandquality.gov.au/resources/comprehensive-care-element-2-identifying-goals-care-tips-clinicians>>

PROMs and PREMs

Find out more

The Australian Real World Cancer Evidence Network (Pan Cancer Initiative) Aims to optimise cancer care delivery and help achieve even better outcomes for all Australians by integrating patient-reported outcome and experience measures (PROMs and PREM), collectively referred to as patient-reported measures (PRMs), across the cancer care sector. <hyperlink <https://www.pancancer.movember.com/>>

ESMO Clinical Practice Guideline: The role of patient-reported outcome measures in the continuum of cancer clinical care Provides recommendations on the role of PROMs during the care of patients with cancer from the start of active treatment during follow-up to the end of life <hyperlink <https://pubmed.ncbi.nlm.nih.gov/35462007/>> Di Maio et al 2022

Australian Hospital Patient Experience Question Set can be used to assess patient experiences of treatment and care in a private or public hospital <hyperlink

<https://www.safetyandquality.gov.au/resources/australian-hospital-patient-experience-question-set-ahpeqs>>

Integrating Patient-Reported Outcomes Into the Care of People With Advanced Cancer – A Practical Guide Lai Kwon ASCO Educational Book 2024 <hyperlink <https://pubmed.ncbi.nlm.nih.gov/38788184/>>

Quality and safety standards



Find out more

Australian Council on Health Standards Help hospitals conform to appropriate standards <hyperlink <https://www.achs.org.au/>>

Survivorship Care for People Affected by Advanced or Metastatic Cancer: MASCC-ASCO Standards and Practice Recommendations Hart et al 2024 <hyperlink <https://pubmed.ncbi.nlm.nih.gov/38684036/>> Endorsed by COSA⁷³

Palliative Care Australia National Palliative Care Standards Provide a nationally consistent framework to guide health and care services in delivering safe, effective palliative care centred on the needs of people with life-limiting illness and their families and carers <hyperlink <https://palliativecare.org.au/national-palliative-care-standards/>>

Treatment protocols



Find out more

eviQ: Cancer treatments online Free resource providing evidence-based, consensus-driven cancer treatment protocols and information <hyperlink <https://www.eviq.org.au/>>

Cancer data



Find out more

Cancer Australia National Cancer Data Framework Sets the direction for collecting, managing and using cancer data across Australia, driving advances in prevention, diagnosis, treatment and research, strengthening outcomes for people affected by cancer. <hyperlink <https://www.canceraustralia.gov.au/key-initiatives/national-cancer-data-framework>>

Cancer Australia National Cancer Control Indicators One-stop, interactive cancer data website, providing high quality data which allows for monitoring and reporting of national trends over time, to provide the knowledge to guide national cancer control in Australia <hyperlink <https://ncci.canceraustralia.gov.au/>>

Australasian Association of Cancer Registries Provides links to state and territory based registries for cancer data <hyperlink <https://www.aihw.gov.au/reports-data/health-conditions-disability-deaths/cancer/aacr>>

Kulay Kalingka Aboriginal and Torres Strait Islander Cancer Control Indicators Data on cancer attitudes, beliefs and experiences from an Aboriginal and Torres Strait Islander perspective from a study of over 2,000 adults about beliefs, attitudes, and experiences of cancer. <hyperlink <https://ncci.canceraustralia.gov.au/aboriginal-and-torres-strait-islander-cancer-control-indicators>>

Australian Cancer Atlas An online, interactive platform from Cancer Council Queensland showing how measures relating to cancer diagnosis, survival, recognised risk factors, screening, and selected hospital treatment vary by small geographical areas across Australia <hyperlink <https://atlas.cancer.org.au/>>

Australian Childhood Cancer Statistics Online An interactive platform that provides the latest statistics on childhood cancer incidence, mortality and survival for the most common cancers among children under the age of 15 in Australia <hyperlink <https://cancerqld.org.au/research/explore-our-projects/australian-childhood-cancer-statistics-online/>>

Networked care

Find out more

Australian Comprehensive Cancer Network Connects cancer services and multidisciplinary health professionals across public and private health systems in Australia with Comprehensive Cancer Centres (CCCs) with the aim of addressing disparities in cancer outcomes and experience <hyperlink <https://www.canceraustralia.gov.au/key-initiatives/australian-comprehensive-cancer-network>>

Multidisciplinary care

Find out more

Cancer Australia Multidisciplinary care Information about best practice approaches to multidisciplinary cancer care; includes information about multidisciplinary care for people with advanced disease <hyperlink <https://www.canceraustralia.gov.au/clinical-best-practice/multidisciplinary-care>>

Supportive care

Cancer support services

Find out more

Cancer Connect A free national information and support service for people affected by cancer, their families and carers, and the health professionals who support them <Hyperlink <https://connect.cancer.org.au/>>

Cancer Council Australia Helpline (13 11 20) A free national telephone information and support service for people affected by cancer, their families and carers, and the health professionals who support them <Hyperlink <https://www.cancer.org.au/support-and-services/support-services/13-11-20>>

CanTeen Cancer Hub A free service for families impacted by cancer to access practical and emotional support services <hyperlink <https://cancerhub.org.au/>>

McGrath Foundation Cancer Nurses McGrath Cancer Care Nurses support people and their families experiencing cancer throughout treatment, for free and without a referral <hyperlink <https://www.mcgrathfoundation.com.au/about/our-nurses/>>

NGO Specialist support services Non-government organisations funded through the ACNNP to provide information, support and advice to people with particular cancer types <hyperlink <https://www.health.gov.au/our-work/australian-cancer-nursing-and-navigation-program>>

Supportive care screening tools

Find out more

NCCN Distress Thermometer and Problem Checklist A validated screening tool that helps clinicians quickly screen people for psychological distress. <hyperlink www.nccn.org/patients/guidelines/content/PDF/distress-patient.pdf>

COSA-endorsed MASCC COG-IMPACT Tool	A COSA-endorsed self-report tool for assessing the difficulties and unmet supportive care needs arising from cancer-related cognitive impairment, for use in both clinical and research settings <hyperlink https://www.cosa.org.au/advocacy/endorsed-publications/the-mascc-cog-impact/ >
Cancer Survivors' Unmet Needs Measure (CaSUN)	A validated 35-item self-report tool for assessing the unmet supportive care needs of cancer survivors <hyperlink https://pubmed.ncbi.nlm.nih.gov/17177268/ >
WeCan tool	Based on the National Comprehensive Cancer Needs Distress Thermometer and Problem List helps people to identify their level of concern or worry and the causes of these concerns or worries <hyperlink https://wecan.org.au/assess-your-needs/ >
This is me	A tool to help people gather and write down important information to share with their cancer team; designed as part of the OlderCan resource it can be used by people of any age <hyperlink https://wecan.org.au/oldercan/cancer-diagnosis/this-is-me/ >
HEEADSS assessment	Health and psychosocial health assessment for adolescents and young adults <hyperlink https://www.racp.edu.au/docs/default-source/advocacy-library/routine-adolescent-psychosocial-health-assessment.pdf >
CanTeen Adolescent and Young Adult Screening Tools	Included in the Adolescent and Young Adult Oncology Psychosocial Care Manual <hyperlink https://www.canteen.org.au/sites/default/files/2021-11/CanteenYCS_AYAOncoPsychosocialCareManual.pdf >
Supportive Care Needs Assessment Tool for Indigenous People (SCNAT-IP)	An evidence based supportive care needs assessment tool which accommodates the language, customs and culture-specific needs of Indigenous people with cancer <hyperlink https://www.scnatip.org/ >
Malnutrition screening tool	A tool to screen for malnutrition that can be completed by a health professional, patient or carer <hyperlink https://www.petermac.org/patients-and-carers/health-services-for-cancer-patients/allied-health-services/nutrition-and-dietetics/malnutrition-screening-tool >
eviQ Frailty assessment tools	Information about a range of tools available to assess frailty in older people <hyperlink https://www.eviq.org.au/clinical-resources/older-people-with-cancer/4614-frailty-and-cancer >

Protocols and guidelines

 Find out more
eviQ: Cancer treatments online A free resource providing evidence-based, consensus driven cancer treatment protocols and information for use at the point of care <hyperlink https://www.eviq.org.au/ >
eviQ side effects and toxicity management Information about side effects of different side effects of cancer treatments <hyperlink https://www.eviq.org.au/clinical-resources/side-effect-and-toxicity-management >
Multinational Association of Supportive Care in Cancer (MASCC) guidelines Evidence-based clinical practice guidelines created by the MASCC study groups covering a breadth of supportive care needs for people with cancer <hyperlink https://mascc.org/resources/mascc-guidelines/ >

Practical and social support

 Find out more
Health Direct Information about Patient Assisted Travel Schemes Information about schemes available in each State and Territory to provide support for people who may need to travel for treatment <hyperlink https://www.healthdirect.gov.au/travelling-to-your-healthcare-appointment >
Cancer Council Financial costs and assistance Public-facing information about costs of cancer care and available financial support <hyperlink https://www.cancer.org.au/support-and-services/financial-assistance >
Gather My Crew Australian charity providing tools, education and guidance to help people support each other <hyperlink https://www.gathermycrew.org.au/ >
Live Work Cancer Australian organisation supporting individuals and workplaces in delivery of workplaces that are aware of, recognise and support the needs of people with cancer <hyperlink: https://liveworkcancer.org.au/ >

Navigation and care coordination

 Find out more
Australian Government Australian Cancer Nursing and Navigation Program (ACNNP) Multifaceted program of services designed to ensure all people affected by cancer have access to care that is high quality, culturally safe, available across all Australian locations and for all cancer types <hyperlink https://www.health.gov.au/our-work/australian-cancer-nursing-and-navigation-program >
Cancer Connect A free national information and support service for people affected by cancer, their families and carers, and the health professionals who support them <Hyperlink https://connect.cancer.org.au/ >
Cancer Council Australia Helpline (13 11 20) A free national telephone information and support service for people affected by cancer, their families and carers, and the health professionals who support them <Hyperlink https://www.cancer.org.au/support-and-services/support-services/13-11-20 >
CanTeen Cancer Hub A free service for families impacted by cancer to access practical and emotional support services <hyperlink https://cancerhub.org.au/ >
McGrath Foundation Cancer Nurses McGrath Cancer Care Nurses support people and their families experiencing cancer throughout treatment, for free and without a referral <hyperlink https://www.mcgrathfoundation.com.au/about/our-nurses/ >
NGO Specialist support services Non-government organisations funded through the ACNNP to provide information, support and advice to people with particular cancer types <hyperlink https://www.health.gov.au/our-work/australian-cancer-nursing-and-navigation-program >
My Health Record A secure online database that helps with data collection and shared access <hyperlink https://www.digitalhealth.gov.au/initiatives-and-programs/my-health-record >

Communication

Health literacy

 Find out more
Australian Commission on Safety and Quality in Health Care National Statement on Health Literacy – Taking action to improve safety and quality Guide providing information about the importance of health literacy and the part it plays in the provision of safe high-quality care <hyperlink

<https://www.safetyandquality.gov.au/resources/national-statement-health-literacy-taking-action-improve-safety-and-quality>>

Communication in other languages



Find out more

Translating and Interpreting Service Language services for people with limited English proficiency and the organisations that support them <hyperlink: <https://www.tisnational.gov.au/>>

eviQ Education Training for healthcare interpreters on cancer clinical trials Education course for healthcare interpreters involved in talking to patients about cancer clinical trials <hyperlink <https://education.eviq.org.au/clinical-and-community-practice/training-for-health-care-interpreters-on-cancer-clinical-trials>>

Australian Cancer Survivorship Centre Multilingual glossary A glossary of cancer terms in nine different languages designed for professional translators, interpreters and bilingual health professionals working in cancer. <hyperlink: www.petermac.org/multilingualglossary>

Digital communication and AI



Find out more

Clinical Oncology Society of Australia Guidelines for tele-oncology Australian guidelines for delivery of telehealth in cancer care <hyperlink <https://app.magicapp.org/#/guideline/nYYb4n>>

Royal Australasian College of Surgeons Using artificial intelligence in clinical practice position statement Details key principles physicians and trainees should consider when implementing, interacting with and monitoring AI in their clinical practice <https://www.racp.edu.au/docs/default-source/advocacy-library/racp-ai-position-statement-using-ai-in-clinical-practice.pdf?sfvrsn=dda7b21a_6>

Royal Australian College of General Practitioners Conversational artificial intelligence Aims to assist GPs weigh up the potential advantages and disadvantages of using conversational AI in their practice <hyperlink <https://www.racgp.org.au/running-a-practice/technology/artificial-intelligence-ai/conversational-artificial-intelligence>>

Australian Health Practitioner Regulation Agency Meeting your professional obligations when using Artificial Intelligence in healthcare Explains how existing responsibilities in National Boards' codes of conduct apply when practitioners use AI in their practice <hyperlink <https://www.ahpra.gov.au/Resources/Artificial-Intelligence-in-healthcare>>

Australian Commission on Safety and Quality in Health Care AI Clinical Use Guide Guidance for health professionals in using artificial intelligence tools in clinical practice <hyperlink <https://www.safetyandquality.gov.au/sites/default/files/resources/attachments/ai-clinical-use-guide.pdf>>

Communication skills



Find out more

eviQ Education Effective patient communication Series of education resources to support effective communication with people with cancer <hyperlink <https://education.eviq.org.au/pages/effective-patient-communication>>

eviQ Education Effective cultural communication in oncology Interactive Communication Skills Training (CST) program in Australia that specifically targets cultural competence in the oncology setting <hyperlink <https://education.eviq.org.au/courses/effective-cultural-communication-in-oncology>>

eviQ Education Communicating with Aboriginal and Torres Strait Islander patients and carers Six practical, effective and culturally appropriate strategies clinicians and researchers can employ to improve communication with First Nations patients and their carers <hyperlink https://education.eviq.org.au/videos-webinars/videos/communicating-with-aboriginal-and-torres-strait-is >
Vital Talk Evidence-based communication skills training courses <hyperlink https://vitaltalk.org/ >
Pam Mclean Centre Interactive workshops for health professionals focused on improving communication <hyperlink https://pammcleancentre.org/ >

Carer communication

 Find out more
Trio Program Online education programs for oncology clinicians and for patients and family carers, which aim to empower all stakeholders during cancer care <hyperlink https://trio.org.au/about-us >

Research and clinical trials

Clinical trials

 Find out more
Cancer Australia Australian Cancer Trials The Australian Cancer Trials database provides up-to-date information on active clinical trials spanning various cancer types, serving as a valuable resource for both clinicians and patients seeking appropriate trial opportunities. <hyperlink https://australiancancertrials.gov.au/ >
Collaborative clinical trials groups undertake clinical trials for a range of different cancer types <hyperlink https://www.canceraustralia.gov.au/research-grants/support-cancer-clinical-trials/multi-site-collaborative-cancer-clinical-trials >
Australian Commission on Safety and Quality in Healthcare National Clinical Trials Governance Framework and user guide <hyperlink https://www.safetyandquality.gov.au/national-standards/national-clinical-trials-governance-framework >
Cancer Australia Understanding Clinical Trials Information about how cancer clinical trials are run in Australia with links to relevant policies <hyperlink https://www.canceraustralia.gov.au/understanding-clinical-trials >
Australian Clinical Trials Talking to your patients about clinical trials Good practice principles for talking to patients about clinical trials < https://www.australianclinicaltrials.gov.au/healthcare-providers/talking-to-your-patients >

Decentralised trials and teletrials

 Find out more
Australian Tele-Trial program A national program supporting tele-trials with regional coordinating centres that support clinicians, researchers and sponsors set up and run local trials <hyperlink https://www.australianclinicaltrials.gov.au/healthcare-providers/talking-to-your-patients >
Regional trial networks are available supporting clinical trials in regional parts of Australia: • NSW Regional Cancer Research Network <hyperlink https://nswregionalcrcn.com.au/home> • Regional Trials Network Victoria <hyperlink https://regionaltrialsnetwork.org/>

Interpreters and clinical trials



Find out more

eviQ Education Training for healthcare interpreters on cancer clinical trials Education course for healthcare interpreters involved in talking to patients about cancer clinical trials <hyperlink <https://education.eviq.org.au/clinical-and-community-practice/training-for-health-care-interpreters-on-cancer-clinical-trials>>

References

- ¹ Sayani A. [Health Equity in National Cancer Control Plans: An Analysis of the Ontario Cancer Plan](#). Int J Health Policy Manage 2019;8(9):550–556.
- ² Sayani A. [Health Equity in National Cancer Control Plans: An Analysis of the Ontario Cancer Plan](#). Int J Health Policy Manage 2019;8(9):550–556.
- ³ Cancer Australia. Australian Cancer Plan. [Achieving equity](#). Accessed July 2026.
- ⁴ Diversity Council of Australia. [Intersectionality](#). Accessed July 2026
- ⁵ Cancer Australia. [National Cancer Data Framework](#). 2025. Accessed July 2026.
- ⁶ Marcelin J et al. [The Impact of Unconscious Bias in Healthcare: How to Recognize and Mitigate It](#). The Journal of Infectious Diseases 2019; 220(2):S62–S73
- ⁷ Australian and New Zealand College of Anaesthetists & Faculty of Pain Medicine. [Unconscious bias toolkit](#). 2022. Accessed 23 May 2026.
- ⁸ Council of Australian Governments (COAG) [Cultural respect framework 2016–2026 for Aboriginal and Torres Strait Islander Health: a national approach to building a culturally respectful health system](#). National Aboriginal and Torres Strait Islander Health Standing Committee, Australian Health Ministers' Advisory Council, Canberra. 2016
- ⁹ Australian Health Practitioner Regulation Agency. [Shared code of conduct principles](#). Accessed 22 May 2026.
- ¹⁰ Australian Commission for Safety and Quality in Healthcare. [Identifying Aboriginal and Torres Strait Islander People](#). 2020 Accessed July 2026.
- ¹¹ Cancer Council Australia. [Financial cost of cancer](#). Accessed July 2026.
- ¹² Clinical Oncology Society of Australia [COSA Guidelines for Teleoncology](#). Accessed July 2026.
- ¹³ Cancer Australia. [Guide to co-design with Aboriginal and Torres Strait Islander people](#). Accessed July 2026.
- ¹⁴ Tavares J et al. [Age-friendly health care: a systematic review](#). Healthcare 2021;Jan 16;9(1):83.
- ¹⁵ John A Hartford Foundation [Age-Friendly Health Systems Initiative](#) Accessed 22 May 2026
- ¹⁶ Cancer Australia. [Childhood, Adolescent and Young Adult Cancer Roadmap](#). May 2026. Accessed July 2026.
- ¹⁷ Fulton J et al. [Evidence and best practices for trauma-informed care in gynecologic oncology patients](#). Int J Gynecol Oncol 2024; 34(7):1077–1088.
- ¹⁸ Australian Commission on Safety and Quality in Healthcare. [Person-centred care](#). Accessed May 2026.
- ¹⁹ Australian Commission on Safety and Quality in Healthcare. [Person-centred care](#). Accessed 19 May 2026.
- ²⁰ Council of Australian Governments (COAG) [Cultural respect framework 2016–2026 for Aboriginal and Torres Strait Islander Health: a national approach to building a culturally respectful health system](#). National Aboriginal and Torres Strait Islander Health Standing Committee, Australian Health Ministers' Advisory Council, Canberra. 2016

-
- ²¹ Australian Health Practitioner Regulation Agency. [Shared code of conduct principles](#). Accessed 22 May 2026.
- ²² Koczwara et al. [Personalised cancer care in the era of precision medicine](#). AJGP 2021;50(8)
- ²³ Koczwara et al. [Personalised cancer care in the era of precision medicine](#). AJGP 2021;50(8)
- ²⁴ Australian Commission on Safety and Quality in Healthcare. [Making informed choices \(Informed consent\)](#). Accessed 19 May 2026.
- ²⁵ Koczwara et al. [Personalised cancer care in the era of precision medicine](#). AJGP 2021;50(8)
- ²⁶ Marion et al. [Prioritizing patient-centered care in a world of increasingly advanced technologies and disconnected care](#). Semin Radiat Oncol 2024;34(4):452–462
- ²⁷ Royal Australian College of General Practitioners [Shared care model between GP and non-GP specialists for complex chronic conditions](#). 2023. Accessed 19 May 2026
- ²⁸ Washington KT et al. [Defining Family-Centered Cancer Care: A Group Concept Mapping Study](#). Psycho-oncology 2025;Jul;34(7):e70235
- ²⁹ Council of Australian Governments (COAG) [Cultural respect framework 2016–2026 for Aboriginal and Torres Strait Islander Health: a national approach to building a culturally respectful health system](#). National Aboriginal and Torres Strait Islander Health Standing Committee, Australian Health Ministers' Advisory Council, Canberra. 2016
- ³⁰ Australian Commission on Safety and Quality in Healthcare. [Person-centred care](#). Accessed 19 May 2026.
- ³¹ Australian College of Nursing. [Person-centred care](#). Accessed 19 May 2026.
- ³² Rutherford et al. [Time is now to consider how we evaluate person-centred care—the role of patient-reported-outcomes](#). Front Health Serv 2025;5
- ³³ Australian Commission on Safety and Quality in Healthcare. [Making informed choices \(Informed consent\)](#). Accessed 19 May 2026.
- ³⁴ Australian Institute of Health and Welfare. [Health care safety and quality](#). Accessed May 2026.
- ³⁵ Segelov et al. [Developing clinical indicators for oncology: the inaugural cancer care indicator set for the Australian Council on Healthcare Standards](#). MJA 2021;214(11): 528-531
- ³⁶ Cancer Australia [National Cancer Control Indicators](#) Accessed 4 June 2026
- ³⁷ Cancer Australia. [Kulay. Kalingka Aboriginal and Torres Strait Islander Cancer Control Indicators](#). Accessed July 2026.
- ³⁸ Cancer Australia, Australian Government, Movember. [The Australian Real World Cancer Evidence Network \(Pan Cancer Initiative\)](#) Accessed July 2026.
- ³⁹ Cancer Australia, Australian Government, Movember. [The Australian Real World Cancer Evidence Network \(Pan Cancer Initiative\)](#) Accessed July 2026.
- ⁴⁰ Australian Council of Healthcare Standards [Radiation Oncology Clinical Indicators](#) Accessed June 2026
- ⁴¹ Australian Health Practitioner Regulatory Authority. [Telehealth consultations with patients](#). Accessed 20 May 2026.
- ⁴² Zon RT et al. [Telehealth in Oncology: ASCO Standards and Practice Recommendations](#). JCO Oncology Practice 2021.
- ⁴³ Clinical Oncology Society of Australia. [COSA guidelines for teleoncology](#). Accessed 20 May 2026.

-
- ⁴⁴ Radiation Oncology Alliance. [Standards of Practice for Artificial Intelligence in Radiation Oncology v0.1](#) Sydney: ROA; 2024
- ⁴⁵ Royal Australasian College of Physicians. [Using artificial intelligence in clinical practice. Position statement](#). March 2026. Accessed July 2026.
- ⁴⁶ Royal Australian College of General Practitioners. [Conversational artificial intelligence \(AI\)](#). Accessed July 2026.
- ⁴⁷ Australian Health Practitioner Regulatory Agency. [Meeting your professional obligations when using Artificial Intelligence in healthcare](#). Accessed July 2026.
- ⁴⁸ Australian Commission on Safety and Quality in Healthcare. [AI Clinical Use Guide](#). Accessed July 2026.
- ⁴⁹ Australian Health Practitioner Regulatory Agency. [Social media: How to meet your obligations under the national law](#). Accessed 20 May 2026
- ⁵⁰ Cancer Australia. [Principles of multidisciplinary care](#). Accessed 20 May 2026.
- ⁵¹ Williams GJ and Thompson JF. [Management changes and survival outcomes for cancer patients after multidisciplinary team discussion: a systematic review and meta-analysis](#). Cancer Treatment Reviews 2025;139
- ⁵² Cancer Australia. [Principles of multidisciplinary care](#). Accessed 20 May 2026.
- ⁵³ Cancer Australia. [Multidisciplinary care principles for advanced disease](#). Accessed 20 May 2026.
- ⁵⁴ Onifade A et al. [Understanding the effectiveness and quality of virtual cancer multidisciplinary team meetings \(MDTMs\): a systematic scoping review](#). BMC Health Services Research. 2024;24(1):1481.
- ⁵⁵ Cancer Australia. [Working in regional centres](#). Accessed 20 May 2026.
- ⁵⁶ Onifade A et al. [Understanding the effectiveness and quality of virtual cancer multidisciplinary team meetings \(MDTMs\): a systematic scoping review](#). BMC Health Services Research. 2024;24(1):1481.
- ⁵⁷ Al-Hammouri T et al. [Implementation of streamlining measures in selecting and prioritising complex cases for the cancer multidisciplinary team meeting: a mini review of the recent developments](#). Frontiers in Health Services 2024;4
- ⁵⁸ Karas PL et al. [Medicolegal Considerations in Multidisciplinary Cancer Care](#). JTO Clinical and Research Reports 2020;1(4).
- ⁵⁹ Multinational Association of Supportive Care in Cancer (MASCC). [What is supportive care?](#) Accessed July 2026.
- ⁶⁰ Chan RJ et al. [Supportive care 2030 movement: towards unifying ambitions for global excellence in supportive cancer care—an international Delphi study](#). eClinical Medicine 2024;76:102825
- ⁶¹ Cancer Australia. [Cancer Care Navigation in Australia – Policy Context and Opportunities for the Future](#). Sydney: Cancer Australia, 2025.
- ⁶² Cancer Australia. [Cancer Care Navigation in Australia – Policy Context and Opportunities for the Future](#). Sydney: Cancer Australia, 2025.
- ⁶³ Cancer Australia. [Cancer Care Navigation in Australia – Policy Context and Opportunities for the Future](#). Sydney: Cancer Australia, 2025.
- ⁶⁴ Stiefel F et al. [Communication and support of patients and caregivers in chronic cancer care: ESMO Clinical Practice Guideline](#). ESMO Open 2024;9(7):103496
- ⁶⁵ Stiefel F et al. [Communication and support of patients and caregivers in chronic cancer care: ESMO Clinical Practice Guideline](#). ESMO Open 2024;9(7):103496

-
- ⁶⁶ Conley CC et al. [Multiple approaches to enhancing cancer communication in the next decade: translating research into practice and policy](#) Translational Behavioural Medicine 2021;11(11):2018-2032
- ⁶⁷ Cancer Australia [A conceptual framework for cancer care during a pandemic incorporating evidence from the COVID-19 pandemic: Review and update July 2022 – Incorporating evidence review to June 2021](#) 2022
- ⁶⁸ Banbury A et al. [Consumers' experiences, preferences, and perceptions of effectiveness in using telehealth for cancer care in Australia](#) APJCO 2023;19(6):752-761
- ⁶⁹ Dykgraaf SH et al. [Older people, the digital divide and use of telehealth during the COVID-19 pandemic](#) AJGP 2022 Sep;51(9):721-724.
- ⁷⁰ Australian Commission on Safety and Quality in Healthcare. [Understanding health information \(health literacy\)](#). Accessed 20 May 2026.
- ⁷¹ Australian Cancer Trials. [What is a clinical trial?](#) Accessed July 2026.
- ⁷² University of Melbourne. [Which study type is that?](#) Accessed 21 May 2026.
- ⁷³ Hart et al. [Quality Survivorship Care for People Affected by Advanced or Metastatic Cancer: A Clinical Oncology Society of Australia Endorsement of the Joint Multinational Association of Supportive Care in Cancer and American Society of Clinical Oncology Care Standards and Practice Recommendations](#) APJCO 2025 Dec;21(6):581-585